CLINICAL NOTE · SURGICAL CANDIDACY, BIOMETRY & LENS SELECTION

Cataract Pre-operative Assessment & IOL Planning

Document functional impairment, surgical risk factors, biometry and IOL calculation, refractive target and the agreed surgical plan.

Completed by: Surgeon + technician | Use as: Note template | Time: 20–30 min

Patient name	Date of birth	Patient ID / MRN	Date of service	Clinician

01 Indication & functional impairment

Visual symptoms in patient's words

Visual function questionnaire (EYE-09) mean (0–4)

Activities impaired despite best correction

- ☐ Reading
- ☐ Driving — day
- ☐ Driving — night / glare
- ☐ Work tasks
- ☐ Recognising faces
- ☐ TV / screens
- ☐ Hobbies
- ☐ Mobility / falls risk

Patient wishes to proceed: ○ Yes ○ No ○ Undecided

Occupation / visual priorities

02 Risk factors

Ocular

- ☐ Prior LASIK / PRK / RK
- ☐ Amblyopia
- ☐ Glaucoma
- ☐ Pseudoexfoliation
- ☐ Fuchs dystrophy / guttae
- ☐ Small pupil (< 6 mm dilated)
- ☐ Shallow AC
- ☐ Dense / white / brunescent lens
- ☐ Phacodonesis / weak zonules
- ☐ Prior vitrectomy
- ☐ Uveitis
- ☐ ERM / AMD / diabetic macular disease
- ☐ High myopia (AL > 26 mm)
- ☐ Short eye (AL < 22 mm)
- ☐ Ocular surface disease (treat before biometry)
- ☐ Monocular patient

Systemic

- ☐ Tamsulosin / alpha-blocker (current or past) — IFIS risk
- ☐ Anticoagulant / antiplatelet
- ☐ Diabetes
- ☐ Unable to lie flat / back problems
- ☐ Sleep apnoea
- ☐ Claustrophobia / anxiety
- ☐ Dementia / cognitive impairment
- ☐ Hearing impairment / language
- ☐ Latex / iodine allergy

03 Examination

Visual acuity	Distance SC	Distance CC	Pinhole	Near SC	Near CC
OD					
OS					
OU					

Refraction	Manifest Rx	BCVA	Glare BCVA (BAT)	Pupil dilated (mm)	IOP
OD					
OS					

Lens grading	Nuclear (1–4+)	Cortical	PSC	Other (PXF, guttae)
OD				
OS				

Structure	OD	OS
Cornea / endothelium	☐ WNL	☐ WNL
Anterior chamber	☐ WNL	☐ WNL
Iris	☐ WNL	☐ WNL
Macula (OCT)	☐ WNL	☐ WNL
Optic nerve	☐ WNL	☐ WNL
Periphery	☐ WNL	☐ WNL

04 Biometry & IOL calculation

Measurement	OD	OS
Device		
Axial length (mm)		
K1 (D @ axis)		
K2 (D @ axis)		
Corneal cylinder / axis		
Posterior / total K (if measured)		
ACD (mm)		
Lens thickness (mm)		
White-to-white (mm)		
CCT (µm)		
Endothelial cell count (cells/mm²)		
Topography (regular / irregular)		
Formula(s)		
IOL model & A-constant		
IOL power (D)		
Predicted refraction (D)		
Target refraction (D)		
Toric: cyl power / alignment axis		
Surgically induced astigmatism / incision		

> Repeat biometry if AL differs > 0.3 mm between eyes, K difference > 1.0 D, or measurements don't match refraction. Use post-refractive formulas (e.g. Barrett True-K) after LASIK / PRK / RK.

05 Lens selection & counselling

IOL options discussed

☐ Monofocal — distance ☐ Monofocal — near ☐ Monovision (dominant eye distance) ☐ Toric

☐ EDOF ☐ Multifocal / trifocal ☐ Light-adjustable lens

Dominant eye

○ OD ○ OS

Agreed refractive goal

Premium lens out-of-pocket acknowledged

Counselled on

☐ Risks & benefits (EYE-26) ☐ Glasses likely needed for some tasks ☐ Dysphotopsia / halos (EDOF / MF)

☐ Second eye timing ☐ Pre-op drops & schedule ☐ Post-op restrictions

06 Surgical plan

First eye
○ OD ○ OS

Surgery date

Facility

Surgeon

Anaesthesia
☐ Topical / intracameral ☐ MAC sedation ☐ Peribulbar / sub-Tenon ☐ General

Adjuncts
☐ Femtosecond laser ☐ Pupil expansion ring ☐ Trypan blue ☐ Capsular tension ring ☐ MIGS combined ☐ Intracameral antibiotic ☐ Intraoperative aberrometry

Pre-op medications & start date

Anticoagulation plan

Medical clearance / H&P

Post-op care
○ Surgeon ○ Co-managed with optometrist

Co-managing provider

Planned transfer of care date

☐ 66984 Cataract extraction with IOL, standard
☐ 66982 Cataract extraction with IOL, complex
☐ 66991 Standard cataract + aqueous drainage device (MIGS)
☐ 66989 Complex cataract + aqueous drainage device
☐ 92136 Optical biometry with IOL calculation
☐ 76519 A-scan ultrasound with IOL calculation

66982 (complex) requires documentation of a device or technique not used in routine surgery (e.g. iris expansion, capsular dye, CTR, primary posterior capsulorrhexis) or an amblyogenic-age patient. Co-management: surgeon -54, post-op provider -55, with written transfer of care.

Clinician signature
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Clinician name & credentials

Date / time signed